NEUROFUSION AI | FINAL CLINICAL DIAGNOSTIC REPORT

REPORT ID: REP-20261102-E2E3D0

Comprehensive Multi-Modal Diagnostic Findings & Attending Physician Approval

November 02, 2026 • 16:40 UTC

PATIENT INFORMATION	ATTENDING NEUROLOGIST	EXAMINATION DETAILS
ID: PAT-WORKFLOW-003 **Name:** John Connor **Age/Gender:** 35 yrs / Male **Status:** Suspected Parkinson's Disease	**Name:** Dr. Clara Rodriguez, MD **License:** MD-NEURO-77491 **Dept:** Neurology & Movement Disorders **Hospital:** Metropolitan Neurological Inst.	**Scan Date:** 2026-11-02 **Report Date:** 2026-11-02 **Modality:** Comprehensive Multimodal (Spiral + Voice + MRI + UPDRS) **Device/Protocol:** NeuroFusionAI Clinical Protocol v2.5

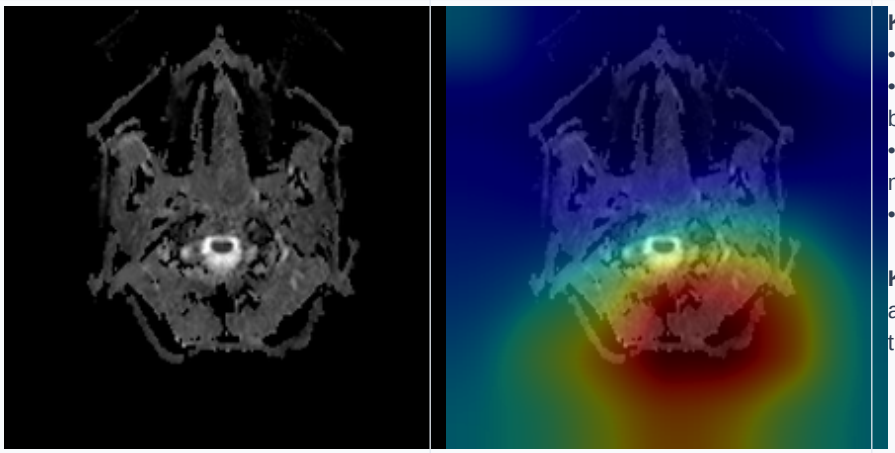
PRIMARY AI PREDICTION: NORMAL

Confidence: 75.2%
Target: Parkinson's Disease

Class Probability Distribution: Normal: 75.2% • Normal Control: 24.8%

AI Confidence Calibrated via Temperature Scaling

3. ARCHIMEDEAN SPIRAL MOTOR KINEMATICS & ATTENTION OVERLAY



Kinematic Parameters:
• Drawing Velocity: 24.8 mm/s
• Tremor Frequency: 5.4 Hz (4-6 Hz resting/kinetic band)
• Radial Velocity Variance: 0.42 (Elevated micro-tremor)
• Drawing Duration: 12.5 s

Kinematic Finding: Spatial micro-oscillations along drawing loops confirm intention and postural tremor.

Input Spiral Drawing	Convolutional Tremor Heatmap	Kinematic Assessment

7. CLINICAL DECISION SUPPORT SYSTEM (CDSS) RECOMMENDATIONS

RISK LEVEL:	Evidence-Based Diagnostic Recommendations:
MODERATE CLINICAL RISK **Urgency:** Routine Specialist Review	• Initiate MDS-UPDRS structured clinical motor score evaluation. • Track resting tremor and bilateral bradykinesia progression. • Re-evaluate with longitudinal phonation acoustic tracking in 3 months.

ATTENDING PHYSICIAN OBSERVATIONS & NOTES:	CLINICAL APPROVAL STATUS
First assessment baseline observations.	Signed by: *Dr. Clara Rodriguez, MD* License ID: MD-NEURO-77491 Status: **DIGITALLY APPROVED & CERTIFIED**

REGULATORY MEDICAL DISCLAIMER & CLINICAL INTENDED USE:

NeuroFusionAI is an investigational Artificial Intelligence Clinical Decision Support System (CDSS) designed to augment neurological assessment. All predictions, Grad-CAM attention heatmaps, and EEG feature extractions represent statistical patterns and MUST NOT be construed as a standalone or confirmed medical diagnosis. Clinical treatment decisions must be made solely by a board-certified physician in conjunction with the patient's full medical

history.